



POLICY

Medication: Formulary and Non-Formulary Prescribing

Scope (Staff):	North Metropolitan Health Service Mental Health Staff
Scope (Area):	North Metropolitan Health Service Mental Health - All Sites

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1. Aim

To outline the principles of prescribing formulary and non-formulary listed medications in the North Metropolitan Health Service Mental Health (NMHS MH) service to ensure optimised individual patient care is provided.

2. Background

The WA Statewide Medicines Formulary (SMF) is a single list of approved medications with appropriate restrictions and guidance for all WA hospitals and Health Services. The purpose of the SMF is to assist the delivery of optimal patient outcomes in an equitable manner.

The SMF is managed and maintained by the WA Drug Evaluation Panel (WADEP). The SMF is informed by evidence-based standards relevant to local practice. The [WA Statewide Medicines Formulary Policy](#) and the [WA Statewide Medicines Formulary Guideline](#) provides overarching governance. It is expected that the vast majority of patients will be adequately treated with the medicines listed on the SMF but it is acknowledged that to provide optimised care for patients, prescribers may on occasion need to utilise medications not listed on the SMF. These medications are termed “non-formulary” medications.

3. Risk

Non-compliance with this policy will impact on the following risk categories:

Emergency Management	☒	Policy & Legislative Compliance	☒
Fiscal Responsibility	☒	Quality of Patient Care	☒
Governance & Accountability	☒		☐

4. Principles

4.1 Formulary Listing

The SMF is a dynamic list intended to reflect the most up to date prescribing practices. The NMHS MH Drugs and Therapeutics Committee (NMHS MH DTC) will provide overarching governance for mental health related formulary applications. Individuals seeking to submit formulary applications should first seek endorsement from the DTC. Once endorsed, submissions for SMF inclusion will then be made by a consultant psychiatrist or nominated DTC member completing the relevant paper or eform available on the [Formulary One](#) home page. The WA Drug Evaluation Panel reviews additions or changes to medications for inclusion on the WA Statewide Medicines Formulary.

4.2 Formulary Prescribing

Prescribers are required to prescribe medicines according to the restrictions and requirements stated in the SMF. Prescribers must consider prescribing SMF medications as first line treatments. Only when these treatments are inappropriate or ineffective may non-formulary or off-label therapies be initiated.

Formulary medications may include those on the Pharmaceutical Benefits Scheme (PBS), PBS restricted and non-PBS medications. Careful consideration should be given to prescribing non-PBS medicines or PBS medicines outside their approved indication as this may have cost implications for the patient. In these situations, the cost implications and availability should be discussed with the patient as this could be a factor in patient medication adherence and treatment outcomes. It should also be noted that items listed on the PBS may not necessarily be included on the SMF. If they are not included on the SMF they are classified as non-formulary medications

The SMF is available via [Formulary ONE intranet](#) or [Formulary ONE internet](#).

4.3 Non-Formulary Prescribing

Medicines termed non-formulary are those medications that:

1. Are not listed on the SMF.
2. Are listed on the SMF but are being used for an indication other than that designated on the SMF.

Once the decision to prescribe a non-formulary medication is taken, the following steps must be undertaken by the prescriber:

- Document in the patient's integrated progress notes the reason for prescribing a non-formulary or off-label medication, why an existing formulary drug is not suitable and the point at which treatment will be reviewed. The treatment must be reviewed regularly and ceased if benefit is not demonstrated.
- If prescribing is off-label, document the discussion with the patient/carer regarding use of an off-label medication and patient/carer consent to treatment in the patients integrated progress notes
- Applications for individual patient approvals (IPA) are to be submitted via the WA Health IPA System WAIPAS.
 - WAIPAS applications may be submitted and endorsed by medical staff, or a pharmacist (for scenarios outlined in practice point 5.1)
 - Applications are assessed and approved according to the following cost stratification

Cost / Medication / Patient / Year	Delegated Approver
Less than \$5000	NMHS MH DTC Chair or delegate
\$5000 and greater	NMHS MH DTC

- Applicants will be notified via email of the assessment outcome

4.4 Complementary and Alternative Medicines (CAMs)

CAMs are defined by the Therapeutic Goods Administration (TGA) as therapeutic agents consisting principally of one or more designated ingredients, each of which has a clearly established identity and/or traditional use. The TGA definition includes vitamins, minerals, nutritional supplements, herbals, certain aromatherapy preparations, homeopathic products and traditional Chinese medicine.

Where a patient wishes to continue using a CAM during an inpatient admission, the CAM will be reviewed for appropriateness and safety by the treating consultant psychiatrist in consultation with the clinical pharmacist.

If continued CAM use is accepted by the treating consultant it will be written on the appropriate medication chart. The CAM will be stored and administered by nursing staff as for any other prescribed medication.

For CAMs not listed on the SMF, patients will organise and pay for the supply of the CAM. Health Service staff will not be involved in obtaining supplies of the CAM. The clinical pharmacist will annotate the medication chart to indicate patients' own supply will be used.

The clinical pharmacist will examine any CAM products prior to use to ensure the product is of a suitable quality for use.

If CAM use is NOT accepted by the treating consultant this will be documented in the patient's notes and the patient will be advised. Any CAM in the patient's possession will be stored in accordance with the local policy on patients' own medication.

5. Practice Points

5.1 Inpatient or Acute Services

Prescribers should always involve a clinical pharmacist in the decision making process to ensure there are no available alternatives with respect to clinical suitability and/or accessibility.

A clinical pharmacist may also complete the IPA in the following circumstances:

- For continuation of treatment prescribed prior to coming into hospital
- For a different formulation of a formulary item
- For a medication prescribed by an external specialist to treat something other than a mental illness

It should also be noted that the pharmacy department may not routinely stock non-formulary medicines. This may result in a delay in provision of the medication. Use of a patient's own medication may be required as an interim measure or where the product is not available from pharmacy.

5.2 Community Clinics

Prescribing should be confined to the treatment of mental illness. Medication to treat simple side effects of psychotropic medications may be prescribed where the side effect is not a medical condition requiring ongoing medical monitoring. This includes medication such as benztropine for extrapyramidal side effects and aperients for medication-induced constipation. Side effects that can be considered as medical conditions requiring ongoing medical monitoring such as metabolic syndrome must be managed by a general practitioner or appropriate specialist.

In circumstances where a prescriber wishes to prescribe medication for a condition other than a mental illness this will always be regarded as non-formulary prescribing. An IPA request must be submitted and approved prior to prescribing.

6. Compliance and Evaluation

- 100% Compliance with this policy in Community Clinics by ongoing audit by NMHS MH Pharmacy.
- 100% Compliance with this policy in Inpatient units by regular chart review of clinical pharmacists.
- Regular review of Non-formulary requests by NMHS MH DTC.

References

[National Safety & Quality Health Service Standards 2021 2nd Edition](#)

[Statewide Medicines Formulary policy \(MP 0077/18\)](#)

[WA Statewide Medicines Formulary Guideline](#)

[Statewide Medicines Formulary](#)



Policy Sponsor	NMHS MH Chief Pharmacist				
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NSQHS Standards Applicable:	☒  Std 1: Clinical Governance		☒  Std 4: Medication Safety		
National Standards for Mental Health Services	☒ Std 8: Governance, leadership and management		☒ Std 10: Delivery of Care ☒ 10.1 Supporting Recovery ☒ 10.4 Assessment and Review ☒ 10.5 Treatment and Support		
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30/06/2014		MHEG	Minutes 47/14
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19/06/2018	V2	Community Liaison Pharmacist	Scheduled review. Changed to reflect use of Formulary One
17/08/2020	V2.1	NMHS MH Chief Pharmacist	Links updated
7 Dec 2021	V3	NMHS MH Chief Pharmacist	Template & Links updated; updated information.
19/05/2022	V3.1	NMHS MH Chief Pharmacist	Addition of information regarding CAMs
03/08/2023	V3.2	NMHS MH Chief Pharmacist	Updated to reflect use of WAIPAS electronic IPA system
16/01/2024	V3.3	NMHS MH Chief Pharmacist	Removal of old non-formulary information. Addition of WAIPAS electronic IPA system

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative ISD1115.

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